NCCP 00322 · VERSION 4

Everolimus + exemestane

Usual starting doses are everolimus 10 mg once daily and exemestane 25 mg once daily; use the doses on your prescription. Both tablets are taken every day continuously. There is no planned rest week. Review benefit and tolerability over time; there is no fixed adjuvant completion date.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Mouth ulcers/stomatitis

☐ Non-infectious pneumonitis

☐ Reduced blood counts

☐ Renal impairment/proteinuria

☐ Joint and muscle pain

☐ Vaginal dryness/sexual symptoms

☐ Fatigue

☐ Infections, including opportunistic infection

☐ High glucose and abnormal lipids

☐ Rash, fatigue and diarrhoea

☐ Impaired wound healing and radiation-related toxicity

☐ Hot flushes and sleep disturbance

☐ Bone thinning and fractures

Discuss the treatment aim, alternatives, benefit and uncertainty, important medicine-specific risks, reproductive precautions, symptom support and planned duration. Confirm the endocrine partner and any ovarian-suppression plan. The timeline does not authorise a restart or certify clinical review.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile; blood urea nitrogen and urinary protein. Blood glucose, total cholesterol and triglycerides; optimise pre-existing metabolic abnormalities. Review lung symptoms, infection, mouth health, all medicines, planned surgery/radiotherapy and exemestane bone risk. FBC, renal/liver profile, total cholesterol and triglycerides every four weeks or as clinically indicated. Fasting glucose as clinically indicated, with closer surveillance for diabetes or increased risk. At reviews assess mouth ulcers, intake, infection, cough/breathlessness, oedema, skin/bowel symptoms and adherence. Disease assessment and imaging follow the consultant plan; new symptoms can bring review forward.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.